



MATERNITY LEAVE BENEFITS

POLICY 3-4

As Amended

INTRODUCTION:

Granting of maternity leave benefits to female employees, married or unmarried, provides an additional incentive for dedicated and faithful performance of their duties and help defray hospital expenses during delivery.

ELIGIBILITY:

All female employees are entitled to maternity leave benefit provided they have rendered at least an aggregate service for at least six (6) months within the last twelve (12) months preceding the expected date of delivery.

POLICIES:

1. All female employees shall be granted maternity leave benefit with pay in accordance with law as follows:
 - a. 60 days for normal delivery, abortion or miscarriage.
 - b. 78 days for cesarian section delivery
2. Maternity benefit may be paid in advance computed as follows:

60 (78) days x daily rate = maternity benefit

This shall include maternity benefit from SSS. The difference between the daily rate computed and the SSS benefit shall be shouldered by the cooperative.

IMPLEMENTATION:

The General Manager is responsible for implementing this policy.

EFFECTIVITY:

This shall take effect upon approval.

Amended per RFP # 94-2115 dated 6-6-03



BENGUET ELECTRIC COOPERATIVE, INC.

Benitez Court, R. Magsaysay Avenue, Baguio City
Tel. Nos. 442-5337/442-2186/ 442-2225/442-6663

21 April 1992

*C.R. [Signature]
4/27/92*

ATTY. FULGENCIO A. VIGARE, JR.
Project Supervisor
BENECO

Sir:

Endorsing to your office for appropriate action amendment to Policy 3-4 on Maternity Leave Benefits.

This is being recommended to conform with SSS' 45 days Maternity Leave as against our Policy of two(2) weeks before delivery and four(4) weeks after delivery or equivalent to 42 days, a difference of three(3) days.

Anticipating for your usual favorable consideration.

Thank you.

Very truly yours,

[Signature]
GERARDO P. VERZOSA
General Manager

AREA COVERAGE:
BAGUIO CITY * BENGUET PROVINCE

RECOMMENDING APPROVAL:

APPROVED:

[Signature]
GERARDO P. VERZOSA
General Manager

218 MEETING, DECEMBER 2, 1978

EXCERPTS FROM THE MINUTES OF THE 218TH MEETING (SPECIAL) OF THE BOARD OF DIRECTORS OF THE BENGUET ELECTRIC COOPERATIVE, INC. (BENECO) HELD AT THE BCF BUILDING, HAMADA SUBDIVISION, BAGUIO CITY ON DECEMBER 2, 1978.

PRESENT: ABUNDIO M. AWAL ----- President
NAPOLEON N. PONCE ----- Vice-President
MAHONG S. MUSCARA ----- Secretary
NESTOR B. FONGWAN ----- Treasurer
LOMINO N. KANITENG ----- P R O.
NICASIO T. ALIPING ----- Director
ANTONIO G. NARCISO ----- Director

TEODORICO P. SANCHEZ ----- General Manager

ABSENT: BASANIO TELLO ----- Director
MANUEL E. GONZALES, JR. ----- Director
ZACARIAS R. SIGNEY ----- Director

RESOLUTION AMENDING POLICY NO. 3 - 4 ON MATERNITY BENEFITS

Resolution No. 87-78

WHEREAS, it has been observed that since January 1, 1978 when SSS assumed payments of maternity benefit, the employees were receiving less than what they would have received based on BENECO Policy No. 3 - 4;

WHEREAS, management recommended that the Board amend said Policy 3 - 4 such that the difference between the benefit an employee would have received based on Policy 3 - 4 and from SSS be paid by the Cooperative; and

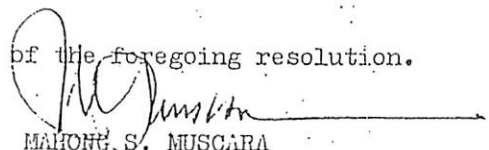
WHEREAS, this body find the recommendation of the management to be highly commendable;

NOW THEREFORE, on motion of Director Muscara duly seconded by Director Kaniteng be it,

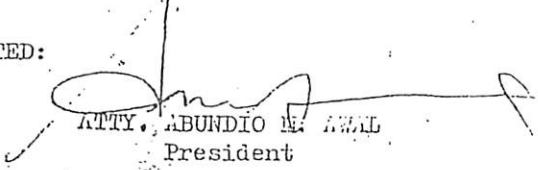
RESOLVED, as it is hereby Resolved to amend policy no. 3 - 4 on maternity benefit such that the difference between the benefits an employee would have received based on Policy 3 - 4 and from SSS be paid by the Cooperative, retroactive to January 1, 1978.

UNANIMOUSLY APPROVED:

I hereby certify to the correctness of the foregoing resolution.


MAHONG S. MUSCARA
Secretary

ATTESTED:


ATTY. ABUNDIO M. AWAL
President



BENGUET ELECTRIC COOPERATIVE, INC.
CAMP ALLEN, BAGUIO CITY
TEL. 21-80

POLICY NO. 3-4

SUBJECT: MATERNITY LEAVE BENEFITS

INTRODUCTION:

Granting of maternity leave benefits to married women employees provides an additional incentive for dedicated and faithful performance of their duties. The Board, realizing that this is a good practice for the cooperative to grant this benefit in conformity with Presidential Decree No. 148, therefore adopts the following:

POLICIES:

1. Married women employees are granted maternity leave with pay in accordance with law, as follows:
 - a. Two (2) weeks prior to the expected date of delivery and another four (4) weeks after normal delivery or miscarriage; *6 weeks or 42 days*
 - b. Maternity leave with pay may be paid in advance, in accordance with cooperative policy, at the rate of sixty percent (60%) of her regular pay;
 - c. Maternity leave is granted to the expectant mother only upon written certification of the Cooperative physician and/or government physician;
 - d. Maternity leave with pay is granted only to married women employees of the Cooperative who have completed six (6) months continuous service.

IMPLEMENTATION:

The General Manager is responsible for implementing this policy No. 3-4.

ADOPTED pursuant to Resolution No. 80 dated November 9, 1974.

ATTESTED:

[Signature]
NICASIO A. ALIPING
Secretary

[Signature]
JACK DULNUAN
President

htc'74

AREA COVERAGE:

BAGUIO CITY • BOKOD • ITOGON • LA TRINIDAD • SABLAN • TUBA • TURLAY



BENGUET ELECTRIC COOPERATIVE
CAMP ALLEN, BAGUIO CITY

POLICY ON MATERNITY LEAVE BENEFITS (Policy No. 3-4)
was amended such that the difference between the be-
nefits an employee would have received based on
Policy 3-4 and from SSS be paid by the Cooperative,
retroactive to January 1, 1978 when the SSS assumed
payments of maternity benefits.

*per Res. No. 87-78 dated
Dec. 2, 1978*

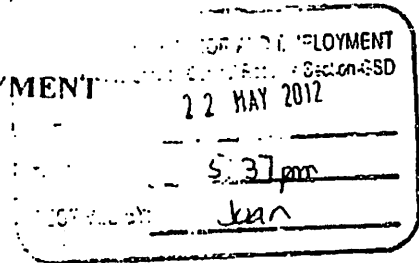
AREA COVERAGE:
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Effectivity: This addendum shall take effect January 1992.

Recommending Approval
Gerardo P. Verzoza
GERARDO P. VERZOZA
Acting General Manager

AREA COVERAGE:
BENGUET PROVINCE

Republic of the Philippines
DEPARTMENT OF LABOR AND EMPLOYMENT
Intramuros, Manila



DEPARTMENT ORDER NO. 112-A
Series of 2012

**AMENDING THE GUIDELINES ON THE IMPLEMENTATION OF THE SPECIAL
LEAVE BENEFIT FOR WOMEN EMPLOYEES IN THE PRIVATE SECTOR**

Section 1. Pursuant to Section 21(B) of the Implementing Rules and Regulations of Republic Act 9710, otherwise known as the "Magna Carta of Women", the provisions of Department Order No. 112, Series of 2011, are hereby amended as follows:

Section 1, paragraph (a) is hereby amended to read as:

Section 1: Definition of terms. - x x x

(a) "Special leave benefit for women" means a female employee's leave entitlement of two (2) months with full pay from her employer based on her gross monthly compensation following surgery caused by gynecological disorders, provided that she has rendered continuous aggregate employment service of at least six (6) months for the last 12 months.

Additional sub-paragraphs are hereby added to Section 1, as follows:

(c) "Gross monthly compensation" means the monthly basic pay plus mandatory allowances fixed by the regional wage boards.

(d) "Two (2) months" means sixty (60) calendar days pursuant to Article 13 of the New Civil Code.

(e) "At least six (6) months continuous aggregate employment service for the last twelve (12) months prior to surgery" means that the woman employee should have been with the company for twelve (12) months, prior to surgery. An aggregate service of at least six (6) months within the said 12-

month period is sufficient to entitle her to avail of the special leave benefit (SLB).

(f) "Employment service" includes absences with pay such as use of other mandated leaves, company granted leaves and maternity leave.

(g) "Competent physician" means a medical doctor preferably specializing in gynecological disorders or is in the position to determine the period of recuperation of the woman employee.

A new provision is hereby introduced as Section 4, to read as:

Section 4. The special leave benefit. – The two (2) months special leave is the maximum period of leave with pay that a woman employee may avail of under RA 9710.

For purposes of determining the period of leave with pay that will be allowed to a woman employee, the certification of a competent physician as to the required period of recuperation shall be controlling.

Section 4 of the previous Guidelines is hereby re-numbered as Section 5, to read as:

Section 5. Availment. – The special leave shall be granted to the qualified employee after she has undergone surgery.

New provisions are hereby added to the Guidelines and subsequent Sections are re-numbered accordingly as follows:

Section 6. Frequency of availment. – A woman employee can avail of the special leave benefit for every instance of surgery due to gynecological disorder for a maximum total period of two (2) months per year.

Section 7. Special leave benefit vis-à-vis SSS sickness benefit. – The special leave benefit is different from the SSS sickness benefit. The former is granted by the employer in accordance with RA 9710, as implemented under this Rules.

Accounting System [UUJET D. ALCISO]

General Ledger | Subsidiary Transactions | Fixed Assets | Cost Distribution | Work Order | Quit | Help

Zimbra: Inbox (889) | Magna Carta Special Leave Ben...

file:///F:/Magna Carta Special Leave Benefit for Women Employees - Working Pinoy.htm

When can you receive your leave pay?
After your surgery, expectedly after you have submitted the medical documents required by your employer.
Some generous employers give the total leave pay before or during the surgery. If they receive a certification from the surgeon certifying the employee's hospital admission and scheduled surgery.

How many times can you avail of this Special Leave Benefit?
You can avail of this leave benefit for every qualified gynecological surgery up to a maximum of 60 calendar days per year.

Can you file for your SSS sickness benefit for the same surgery?
YES
Special Leave Benefit will be paid by your employer.
SSS sickness benefit will be paid by SSS.

Can your employer use your regular leaves to cover your Special Leave Benefit?
NO
Your Special Leave Benefit is different from your regular leaves (e.g. sick leaves, vacation leaves, 5-day special incentive leave, parental leave for solo parents, leave for victims of violence against women).
In fact, if you need an extended leave, you can use your sick leave or other regular leaves in addition to your Magna Carta Special Leave Benefit.

What if your gynecological surgery is performed during your maternity leave?
Your employer will pay the difference between your Special Leave Benefit and your Maternity Leave Benefit.

Can you decline your special leave and convert it to cash?
NO
Unless your company CBA has a provision that covers this.

Which agency monitors the enforcement of the Magna Carta of Women's Special Leave Benefit?
The DOLE Regional Office in your area.

For women employees in the public sector, are there specific rules that apply to them?
Yes, there are. These are the following:
1. Apply for the special leave benefit at least 5 days prior to the scheduled date of surgery. Use the Civil Service Form No. 6.
2. After your surgery, submit the following documents:
- Medical certificate describing the operative technique used, duration of the surgery and

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Modified: 7/28
Size: 226

It is granted to a woman employee who has undergone surgery due to gynecological disorder. The SSS sickness benefit, on the other hand, is administered and given by the SSS in accordance with the SSS law or RA 1161 as amended by RA 8282.

Section 8. Special leave benefit vis-à-vis existing statutory leaves. – The special leave benefit cannot be taken from existing statutory leaves (i.e. 5-day Service Incentive Leave, Leave for victims of VAWC, Parental Leave for Solo Parents). The grant of the special leave benefit under the law is in recognition of the fact that patients with gynecological disorder needing surgery require a longer period for recovery. The benefit is considered an addition to the leave benefits granted under existing laws and should be added on top of said statutory leave entitlements.

If the special leave benefit has already been exhausted, the company leave and other mandated leave benefits may be availed of by the woman employee.

Section 9. Special leave benefit vis-à-vis maternity leave benefit. – Where the woman employee had undergone surgery due to gynecological disorder during her maternity leave, she is entitled only to the difference between the SLB and maternity leave benefit.

Section 10. Crediting of existing or similar benefits. – If there are existing or similar benefits under a company policy, practice or collective bargaining agreement (CBA) providing similar or equal benefits to what is mandated by law, the same shall be considered as compliance, unless the company policy, practice or CBA provides otherwise.

In the event the company policy, practice or CBA provides lesser benefits, the company shall grant the difference.

More liberal existing or similar benefits cannot be withdrawn or reduced by reason of the mandate of RA 9710.

The term "similar or equal benefits" refers to leave benefits which are of the same nature and purpose as that of the SLB.

Section 11. Mode of payment. – The special leave benefit is a leave privilege. The woman employee shall not report for work for the duration of the leave but she will still receive her salary covering said period. The employer, in its discretion, may allow said employee to receive her pay for the period covered by the approved leave before or during the surgery. The computation of her "pay" shall be based on her prevailing salary at the time of the surgery.

Section 12. Non-commutation of the benefit. – The special leave shall be non-cumulative and non-convertible to cash unless otherwise provided by a collective bargaining agreement (CBA).

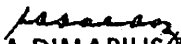
Section 13. Retroactive application. – The woman employee whose leave period for surgery and recuperation due to gynecological disorders after the effectivity of the Magna Carta of Women (RA No. 9710) on 15 September 2009 and before the promulgation of its Guidelines (DO 112-11) on April 6, 2011, was deducted against her sick or vacation leave credits shall be entitled to the restoration of said leave credits and/or payment of appropriate compensation or salary at the time of surgery, as the case may be.

Section 14. Monitoring of compliance. – The DOLE-Regional Office shall be responsible for monitoring compliance as provided herein and related rules and issuances. It shall submit a separate quarterly monitoring report to the Bureau of Working Conditions (BWC), copy furnished the Tripartite Industrial Peace Council (TIPC), for purposes of evaluation.

Section 2. Repealing Clause. – All rules, regulations, circulars and administrative orders inconsistent herewith are repealed or modified accordingly.

Section 3. Effectivity. – This Guidelines shall take effect fifteen (15) days after its publication in a newspaper of general circulation.

Manila, Philippines, 22 MAY, 2012.


ROSALINDA DIMAPILIS BALDOZ
Secretary

**DISSEMINATION OF CIVIL SERVICE COMMISSION (CSC) MEMORANDUM
CIRCULAR NO. 25, S. 2010**

**Availment of the Special Leave Benefits for Women Under R.A. NO. 9710
(An Act Providing for the Magna Carta of Women)**

For the information and guidance of all concerned, enclosed is a copy of Civil Service Commission (CSC) Memorandum Circular No. 25, s. 2010 dated December 1, 2010 entitled "Availment of the Special Leave Benefits for Women Under R.A. No. 9710 (An Act Providing for the Magna Carta of Women)," which is self-explanatory.

This Memorandum Circular, if properly implemented, will give female teachers/personnel enough courage to undergo surgical operation and prevent further worsening of their condition if it remains unattended. It will also assure that their salaries and leave credits remain unaffected. The said Memorandum Circular is retroactive effective September 15, 2009 or fifteen (15) days after the publication of the Magna Carta of Women. Female teachers/personnel who availed of this kind of leave, and have undergone surgery caused by gynecological disorder, will have their leave credits restored and if without pay, the salaries deducted should be refunded.

CIVIL SERVICE COMMISSION MC No. 25, s. 2010

MEMORANDUM CIRCULAR

**TO: ALL HEADS OF CONSTITUTIONAL BODIES; DEPARTMENTS; BUREAUS
AND AGENCIES OF THE NATIONAL GOVERNMENT; LOCAL GOVERNMENT
UNITS; GOVERNMENT-OWNED AND CONTROLLED CORPORATIONS WITH
ORIGINAL CHARTER; AND STATE COLLEGES AND UNIVERSITIES**

**SUBJECT: Guidelines on the Availment of the Special Leave Benefits for Women
Under R.A. 9710 (An Act Providing for the Magna Carta of Women):**

Pursuant to CSC Resolution No. 1000432 dated November 22, 2010, the following Guidelines on the Availment of the Special Leave Benefits for Women Under R.A. 9710 (An Act Providing for the Magna Carta of Women) are hereby prescribed for the guidance of all concerned:

1.0 Purpose

1.1 To provide further guidelines on the availment of special leave benefits for qualified female public sector employees who have undergone surgery caused by gynecological disorders' pursuant to the provisions and implementing rules and regulations of the Magna Carta of Women.

1.2 To ensure uniform interpretation and implementation of the grant of the special leave

benefits for women and ensure that the availment of the same ultimately upholds the objectives of the Law.

2.0 Guidelines on Entitlement to the Special Leave Benefits for Women

2.1 Any female public sector employee², regardless of age and civil status, shall be entitled to a special leave of a maximum of two months with full pay based on her gross monthly compensation³, provided she has rendered at least six (6) months aggregate service in any or various government agencies for the last twelve (12) months prior to undergoing surgery for gynecological disorders.

2.1.1 The special leave may be availed for every instance of gynecological disorder requiring surgery for a maximum period of two (2) months per year.

2.2 Generally, availment of the said special leave benefits shall be in accordance with the attached **List of Surgical Operations for Gynecological Disorders** 4 (Annex A), which reflects, among others, the estimated periods of recuperation from surgery due to the specific gynecological disorder.

2.2.1 The said List of Surgical Operations for Gynecological Disorders reflects, among others a classification of the Procedure based on the patient's estimated period of recuperation, defined as follows:

Classification of the Procedure based on the Patient's Estimated Period of Recuperation if without concomitant medical problems	Description
Minor	Surgical Procedures requiring a maximum period of recuperation of two (2) weeks
Major	Surgical Procedures requiring a minimum period of recuperation of three (3) weeks to a maximum period of two (2) months

2.2.2 Other Surgical Operations for Gynecological Disorders which are not found in Annex A of this Guidelines may be allowed subject to certification of a competent medical authority and submission of other requirements provided under item 3.1 hereof.

2.3 The earned leave credits may be used for preparatory procedures and/or confinement prior to the surgery. Moreover, should the period of recuperation after the surgery exceed two (2) months, the female official/employee may use her earned sick leave credits for the same. If the sick leave credits have been exhausted, the vacation leave credits may be used pursuant to Section 56 of the Omnibus Rules on Leave.

2.4 This special leave benefit is non-cumulative and not convertible to cash.

3.0 Procedure for Availment of the Special Leave Benefits for Women

3.1 The application for the special leave benefit shall be made through the Civil Service Form No. 6 (CS Form 6) signed by the employee and approved by the proper signing

authorities.

3.2 The CS Form 6 shall be accompanied by a medical certificate filled out by the proper medical authorities, e.g. the attending surgeon accompanied by a clinical summary reflecting the gynecological disorder 5 which shall be addressed or was addressed by the said surgery; the histopathological report; the operative technique used for the surgery; the duration of the surgery including the peri-operative period (period of confinement around surgery); as well as the employee's estimated period of recuperation for the same.

3.3 The application for the special leave benefits may be applied for in advance, that is, at least five (5) days prior to the scheduled date of the gynecological surgery that will be undergone by the employee. The rest of the requirements specified in Item 3.2 shall be attached to the medical certificate upon the employee's return to work under Item 3.5 of this Guidelines.

The advance notice for taking such leave would give the proper authorities ample time and means to prevent the disruption of the operations of the work unit during the absence of the employee and to address the exigency of services of the office.

3.4 In instances when a qualified female employee undergoes an emergency surgical procedure, the said leave application shall be filed immediately upon the employee's return from such leave, also following the procedure set forth in Item 3.1 and 3.2 of this Guidelines.

3.5 Upon the employee's return to work, she shall also present a medical certificate signed by her attending surgeon that she is physically fit to assume the duties of her position.

4.0 Responsibilities of the Agency Head

4.1 The agency head shall ensure that the aforecited guidelines are enforced in one's agency as a mechanism in order that female employee's right to proper reproductive health care is ensured.

4.2 The agency head shall promote reproductive health care awareness and wellness program for its employees through proactive measure/s such as conduct of annual physical/medical check-up, information campaign on maintaining proper reproductive health care; issuance of health advisories; distribution of educational reading materials and conduct of fora relative to the same.

5.0 Effectivity

These Guidelines shall take effect retroactively starting September 15, 2009 or fifteen (15) days after the publication of the Magna Carta of Women.

Government officials and employees covered in these Guidelines whose periods of surgery and recuperation due to gynecological disorders after the effectivity⁶ of the Magna Carta of Women and before the promulgation of these Guidelines were deducted against their sick or vacation leave credits can have the said leave credits restored and/or appropriate gross compensation paid, as the case may be.

Undersigned
FRANCISCO T. DUQUE III, MD, MSc
Chairman

'Gynecological disorders refer to disorders that would require surgical procedures such as, but not limited to dilatation and curettage and those involving female reproductive organs such as the vagina, cervix, uterus, fallopian tubes, ovaries, breast, adnexa and pelvic floor, as certified by a competent physician. For purposes of the Act and these Rules and Regulations, gynecological surgeries shall also include hysterectomy, ovariectomy, and mastectomy. (Item M, Section 7, Rule II of the Implementing Rules and Regulations of RA 7910 Otherwise known as The Magna Carta for Women).

2 "Employee" refers to public officials in the career and non-career service who are employed in the civil service. Those without an employer-employee relationship such as those on Contracts of Service or Job Orders are not covered by this Guidelines.

3 "Gross Monthly Compensation" refers to the monthly basic pay plus mandatory allowances fixed by Law given in support of a public sector employee's monthly cost of living expenses in addition to salaries such as, Personal Economic Relief Allowance (PERA). Said Gross Monthly Compensation shall exclude, however, allowances and other forms of compensation such as RATA and the like that an employee is regularly entitled to by virtue of his/her performance of the functions of his/her position, all in accordance with pertinent rules and regulations of the Department of Budget and Management (DBM).

° Annex A of this Guidelines refers to The List of Surgical Operations for Gynecological Disorders formulated by a Technical Working Committee composed of Obstetrician-Gynecologists, Surgeons, and Medical Doctors from the University of the Philippines-Philippine General Hospital (UP-PGH), the Department of Health's (DOH) Quirino Memorial Medical Center, the Philippine Health Insurance Corporation (Phi/Health) with the support of the Philippine Obstetrical and Gynecological Society, Inc (POGS) and the Philippine College of Surgeons (PCS) and the DOH's Dr. Jose Fabella Memorial Hospital. The said List reflects the type of surgical procedure for the gynecological disorder; the disease being addressed by the said surgical procedure; as well as the classification or type of procedure to be undertaken/undertaken based on the patient's estimated period of recuperation (if without concomitant medical problems) as agreed upon in the discussion and inputs of the members of the aforementioned Technical Working Committee.

LIST OF SURGICAL OPERATIONS FOR GYNECOLOGICAL DISORDERS

Vulva, Perineum, and Introltus

Procedure	Disease	Classification*
Incision and drainage of vulvar or perineal abscess/masses	Vulvar or perineal abscess	Minor
Incision and drainage of Bartholin's gland abscess	Bartholin's gland abscess	Minor
Marsupialization of Bartholin's gland cyst	Bartholin's gland cyst	Minor
Lysis of labial adhesions	Labial adhesions	Minor
Blopsy of vulvar or perineal masses	Vulvar warts Vulvar Masses	Minor
Electrocautery of vulvar warts	Vulvar Warts	Minor
Vulvectomy simple; partial or complete	Vulvar Masses	Major
Vulvectomy, radical, partial; <i>w/ unilateral inguofemoral lymphadenectomy</i> <i>w/ bilateral inguofemoral lymphadenectomy</i>	Vulvar carcinoma	Major
Vulvectomy, radical, complete; <i>w/ unilateral inguofemoral lymphadenectomy</i> <i>w/ bilateral inguofemoral lymphadenectomy</i>	Vulvar carcinoma	Major
Vulvectomy, radical, complete, w/ inguofemoral, iliac, and pelvic lymphadenectomy	Vulvar carcinoma	Major
Partial hymenectomy or revision of hymenal ring	Imperforate hymen	Minor
Hymenotomy, simple incision	Imperforate hymen	Minor
Excision of Bartholin's gland or cyst	Bartholin's gland cyst/abscess	Minor

Vagina

Procedure	Disease	Classification
Biopsy of vaginal mucosa and/or masses	Vaginal warts, vaginal masses	Minor
Colpocleisis (Le Fort type)	Uterine prolapse	Major
Excision of vaginal septum	Transverse vaginal septum	Minor
Excision of vaginal cyst or tumor	Vaginal cyst Vaginal masses	Minor
Insertion of uterine tandems and/or vaginal ovoids for clinical brachytherapy	Cervical or endometrial cancer	Minor
Colporrhaphy, suture of injury of vagina (nonobstetrical)	Trauma	Minor
Colpoperineorrhaphy, suture of injury of vagina and/or perineum (nonobstetrical)	Trauma	Minor
Plastic operation on urethral sphincter, vaginal approach (eg, Kelly urethral plication)	Urethrocele	Minor
Plastic repair of urethrocele	Urethrocele	Minor
Anterior and/or posterior colporrhaphy	Cysto+/-urethrocele	Major
Anterior and/or posterior colporrhaphy, w/ or w/o perineorrhaphy	Rectocele	Major
Combined anteroposterior colporrhaphy;	Cystocele with rectocele	Major
w/ enterocele repair	Pelvic organ prolapse	Major
Repair of enterocele, vaginal approach	Pelvic organ prolapse	Major
Repair of enterocele, abdominal approach	Pelvic organ prolapse	Major
Colpopexy, abdominal approach	Pelvic organ prolapse	Major
Sacrospinous ligament fixation for prolapse of vagina	Pelvic organ prolapse	Major
Prespinous on Iliococcygeal ligament fixation	Pelvic organ prolapse	Major
Paravaginal defect repair (including repair of cystocele, stress urinary incontinence, and/or incomplete vaginal prolapse)	Pelvic organ prolapse	Major
Sling operation for stress incontinence (eg, fascia or synthetic)	Urinary stress incontinence	Major
Burch calposuspension/retropubic urethropexy	Urinary stress incontinence	Major
Pereyra procedure, including anterior colporrhaphy	Urinary stress incontinence	Major

Procedure	Disease	Classification
Repair of rectovaginal fistula; vaginal or transanal approach	Rectovaginal fistula	Major
abdominal approach	Rectovaginal fistula	Major
abdominal approach, w/ concomitant colostomy	Rectovaginal fistula	Major
Repair of urethrovaginal fistula;	Urethrovaginal fistula	Major
w/ bulbocavernosus transplant		Major
Repair of vesicovaginal fistula; vaginal approach	Vesicovaginal fistula	Major
transvesical and vaginal approach		
Removal of impacted vaginal foreign body under anesthesia	Retained foreign body	Minor
Laparoscopy, surgical, colpexy (suspension of vaginal apex)	Pelvic organ prolapse	Major
Colposcopy (Vaginoscopy)	Vaginal Intraepithelial lesions	Minor
Colposcopy; w/ biopsy(s) of the cervix and/or endocervical curettage	Vaginal and cervical intraepithelial lesions	Minor
Colposcopy; w/ loop electrode excision procedure of the cervix	Cervical Intraepithelial lesions	Minor

Cervix

Procedure	Disease	Classification
Cervical Biopsy, single or multiple, or local excision of lesion, w/ or w/o fulguration	Cervical pathology	Minor
Cauterization of cervix; any method	Cervical warts	Minor
Conization of cervix, w/ or w/o fulguration, w/ or w/o dilation and curettage, w/ or w/o repair; cold knife or laser loop electrode excision	Cervical intraepithelial neoplasia	Minor
Trachelectomy (cervicectomy), amputation of cervix	Cervical masses	Major
Excision of cervical stump, abdominal approach; w/ or w/o pelvic floor repair	S/p subtotal hysterectomy	Major
Excision of cervical stump, vaginal approach; w/ anterior and/or posterior repair w/ repair of enterocele	S/p subtotal hysterectomy +/- pelvic organ prolapse	Major

Procedure	Disease	Classification
Trachelorrhaphy, plastic repair of uterine cervix, vaginal approach	Cervical lacerations	Minor
Cerclage of cervix, during pregnancy; vaginal abdominal	Cervical incompetence	Major
Hysterorrhaphy of ruptured uterus	Cervical incompetence	Major

Uterus

Procedure	Disease	Classification
Endometrial sampling (biopsy) w/ or w/o endocervical sampling (biopsy), w/o cervical dilation, any method	Uterine pathologies	Minor
Dilation and curettage	Uterine pathologies	Minor
Vaginal Myomectomy, excision of fibroid tumor of uterus, single or multiple	Uterine pathologies	Minor
Myomectomy, excision of fibroid tumor of uterus, single or multiple ; abdominal approach	Uterine pathologies	Major
Total abdominal hysterectomy (corpus and cervix), w/ or w/o removal of tube(s), w/ or w/o removal ovary(s);	Uterine, ovarian and fallopian pathologies	Major
Supracervical abdominal hysterectomy (subtotal hysterectomy), w/ or w/o removal of tube(s), w/ or w/o removal of ovary(s)	Uterine, ovarian and fallopian tube pathologies	Major
Total abdominal hysterectomy, including partial vaginectomy, w/ para-aortic and pelvic lymph node sampling, w/ or w/o removal of tube(s), w/ or w/o removal removal of ovary(s)	Uterine, ovarian, fallopian tube malignancies	Major
Radical abdominal hysterectomy, w/ bilateral total pelvic lymphadenectomy and para-aortic lymph node sampling (biopsy), w/ or w/o removal of tube(s), w/ or w/o removal of ovary(s)	Uterine, ovarian malignancies	Major

Procedure	Disease	Classification
Pelvic exenteration for gynecologic malignancy, w/ total abdominal hysterectomy or cervicectomy, w/ or w/o removal of tube(s), w/ or w/o removal of ovary(s), w/ removal of bladder and ureteral transplantations, and/or abdominoperineal resection of rectum and colon and colostomy, or any combination thereof	Uterine, ovarian, fallopian tube malignancies	Major
Vaginal hysterectomy;	Pelvic organ prolapse/stress urinary incontinence	Major
w/ removal of tube(s), and/or ovary(s)	Pelvic organ prolapse/stress urinary incontinence	Major
w/ removal of tube(s), and/or ovary(s), w/ repair of enterocele	Pelvic organ prolapse/stress urinary incontinence	Major
w/ colpo-urethrocystopexy (Marshall-Marchetti-Krantz type, Pereyra type, w/ or w/o endoscopic control)	Pelvic organ prolapse/stress urinary incontinence	Major
w/ repair of enterocele	Pelvic organ prolapse/stress urinary incontinence	Major
Vaginal hysterectomy, w/ total or partial colectomy;	Pelvic organ prolapse/stress urinary incontinence	Major
w/ repair of enterocele	Pelvic organ prolapse/stress urinary incontinence	Major
Vaginal hysterectomy, radical (Schauta type operation)	Pelvic organ prolapse with associated cervical cancer	Major
Uterine suspension, w/ or w/o shortening of round ligaments, w/ or w/o shortening of sacrouterine ligaments;	Pelvic organ prolapse	Major
Hysterorrhaphy, repair of ruptured uterus (nonobstetrical)	Non-obstetrical uterine rupture (e.g. trauma)	Major
Hysteroplasty, repair of uterine anomaly (Strassman type)	Mullerian anomalies, eg. Septate uterus	Major
Laparoscopy, surgical, myomectomy, excision; intramural myomas and/ or removal of surface myomas	Uterine pathologies	Major
Laparoscopy surgical, with vaginal hysterectomy; with removal of tube(s) and/ or ovary(s)	Uterine pathologies	Major

Procedure	Disease	Classification
Hysteroscopy, diagnostic	Uterine pathologies	Minor
Hysteroscopy, surgical; with sampling (biopsy) of endometrium and / or polypectomy, with or without D & C	Uterine pathologies	Minor
with lysis of intrauterine adhesions (any method)	Uterine pathologies	Minor
with division or resection of intraterine septum (any method)	Uterine pathologies	Minor
with removal of leiomyomata	Uterine pathologies	Minor
with removal of impacted foreign body	Uterine pathologies	Minor
with endometrial ablation (e.g., endometrial resection, electrosurgical ablation thermoablation)	Uterine pathologies	Minor
with bilateral fallopian tube cannulation to induce occlusion by placement of permanent implants	Uterine pathologies	Minor
Laparoscopy, surgical; with lysis of adhesions (salpingolysis)	Fallopian tube pathologies	Minor
with removal of adnexal structures (partial or total oophorectomy and/ or salpingectomy)	Fallopian tube pathologies	Major
with fulguration or excision of lesions of the ovary, pelvic viscera, or peritoneal surface by any method	Fallopian tube pathologies	Minor
with fulguration of oviducts (with or without transection)	Fallopian tube pathologies	Minor
with occlusion of oviducts by device (e.g., band, clip, or Falope ring)	Fallopian tube pathologies	
with fimbrioplasty	Fallopian tube pathologies	Major
with salpingostomy (salpingoneostomy)	Fallopian tube pathologies	Major

Oviduct

Procedure	Disease	Classification
Tubal Reanastomosis	Fallopian tube pathologies	Major
Salpingectomy, complete or partial, unilateral or bilateral	Fallopian tube pathologies	Major
Salpingo-oophorectomy, complete or partial, unilateral or bilateral	Fallopian tube and ovarian pathologies	Major

Procedure	Disease	Classification
Laparotomy, for staging or restaging of ovarian malignancy ("second look"), w/ or w/o omentectomy, peritoneal washing, biopsy of abdominal and pelvic peritoneum, diaphragmatic assessment w/ pelvic and limited para-aortic lymphadenectomy	Ovarian cancer	Major
Ovariolysis	Lysis of Adhesions	Major

Breast Procedures

Procedure	Disease	Classification*
Puncture aspiration of cyst of breast	Simple breast cyst, Fibrocystic change	Minor
Mastotomy w/ exploration or drainage of abscess, deep	Breast abscess/Mastitis	Minor
Biopsy of breast; needle core, fine needle aspiration	Breast mass, benign or malignant	Minor
Excision of lactiferous duct fistula	Intraductal Papilloma	Minor
Excision of cyst, fibroadenoma, or other benign breast masses	Fibroadenoma, Fibrocystic change	Minor
Incision/Excision biopsy	Benign breast masses or breast cancer	Minor
Wide excision	Phyllodes tumor, Ductal carcinoma in-situ, Lobular carcinoma in-situ	Major
Total Mastectomy	Phyllodes tumor, Ductal carcinoma in-situ, Lobular carcinoma in-situ	Major
Mastectomy, subcutaneous	Silicone Mastitis	Major
Radical/Modified Radical Mastectomy	Breast cancer	Major
Lumpectomy/quadrantectomy, axillary node dissection	Breast cancer	Major
Lumpectomy, sentinel node biopsy +/- axillary node dissection	Breast cancer	Major
Breast reconstruction with latissimus dorsi flap, with or without prosthetic implant	Breast cancer, Phyllodes tumor, Ductal carcinoma in-situ, lobular carcinoma in-situ (after mastectomy)	Major
Breast reconstruction with free flap	Breast cancer, Phyllodes tumor, Ductal carcinoma in-situ, lobular carcinoma in-situ (after mastectomy)	Major
Breast reconstruction with other technique	Breast cancer, Phyllodes tumor, Ductal carcinoma in-situ, lobular carcinoma in-situ (after mastectomy)	Major

Procedure	Disease	Classification
Lysis of adhesions (salpingolysis)	Fallopian tube pathologies	Major
Fimbrioplasty	Fallopian tube pathologies	Major
Salpingostomy (salpingoneostomy)	Fallopian tube pathologies	Major
Transcervical introduction of fallopian tube catheter for diagnosis and/or re-establishing patency (any method) w/ or w/o hysterosalpingography	Fallopian tube pathologies	Major

Ovary

Procedure	Disease	Classification
Aspiration of ovarian cyst(s), unilateral or bilateral ; vaginal approach	Ovarian cyst	Minor
Drainage of ovarian abscess; vaginal approach	Tuboovarian abscess	Minor
Ovarian cystectomy, unilateral or bilateral	Benign ovarian cysts (e.g. endometriotic cyst, dermoid cyst, serous cystadenoma, mucinous cystadenoma)	Major
Oophorectomy, partial or total, unilateral or bilateral;	Benign ovarian cysts	Major
<i>for ovarian malignancy, w/ para-aortic and pelvic lymph node biopsies, peritoneal washings, peritoneal biopsies, diaphragmatic assessments, w/ or w/o salpingectomy(s), w/ or w/o peritoneal biopsies, diaphragmatic assessments, w/ or w/o salpingectomy(s), w/ or w/o omentectomy</i>	Ovarian cancer	Major
<i>Resection of ovarian malignancy w/ bilateral salpingo-oophorectomy and omentectomy;</i>	Ovarian cancer	Major
<i>w/ total abdominal hysterectomy, pelvic and limited para-aortic lymphadenectomy</i>	Ovarian cancer	Major
<i>w/ radical dissection for debulking</i>	Ovarian cancer	Major

Procedure	Disease	Classification*
Breast reconstruction with transverse rectus abdominis myocutaneous flap (TRAM)	Breast cancer, Phyllodes tumor, Ductal carcinoma in-situ, lobular carcinoma in-situ (after mastectomy)	Major

Legend:

**Classification refers to the estimated period of one's recuperation after surgery, if without concomitant medical problems.*

*Minor - pertains to one's estimated period of recuperation requiring a maximum of two weeks
Major - pertains to one's estimated period of recuperation more than three weeks to two months*

Republic of the Philippines
DEPARTMENT OF LABOR AND EMPLOYMENT
Intramuros, Manila

DEPARTMENT OF LABOR AND EMPLOYMENT
Office of the Secretary, Department of Labor and Employment
DATE: 11 MAR 2011
TIME: 1:52pm
RECEIVED BY: JUAN

DEPARTMENT ORDER NO. 112 - 11
Series of 2011

**GUIDELINES GOVERNING THE IMPLEMENTATION OF THE SPECIAL LEAVE
BENEFITS FOR WOMEN EMPLOYEES IN THE PRIVATE SECTOR**

Pursuant to Section 21 (B) of the Implementing Rules and Regulations of Republic Act 9710, otherwise known as the "Magna Carta of Women", the following guidelines relative to the application of the special leave benefits for women is hereby issued for the guidance and compliance of all concerned.

Section 1. Definition of terms.-As used in these Rules, the following terms shall mean:

- (a) **Special leave benefits for women** refers to a female employee's leave entitlement of two (2) months with full pay from her employer based on her gross monthly compensation following surgery caused by gynecological disorders, provided that she has rendered continuous aggregate employment service of at least six (6) months for the last 12 months. This two-month leave is in addition to leave privileges under existing laws.
- (b) **Gynecological disorders**, refers to disorders that would require surgical procedures such as, but not limited to, dilatation and curettage and those involving female reproductive organs such as the vagina, cervix, uterus, fallopian tubes, ovaries, breast, adnexa and pelvic floor, as certified by a competent physician. For purposes of the Act and the Rules and Regulations of this Act, gynecological surgeries shall also include hysterectomy, ovariectomy, and mastectomy.

Section 2. Conditions to entitlement of special leave benefits. - Any female employee, regardless of age and civil status, shall be entitled to a special leave, provided she has complied with the following conditions:

- (a) She has rendered at least six (6) months continuous aggregate employment service for the last twelve (12) months prior to surgery;
- (b) She has filed an application for special leave in accordance with Section 3 hereof.
- (c) She has undergone surgery due to gynecological disorders as certified by a competent physician.

Section 3. Application for special leave. - The employee shall file her application for leave with her employer within a reasonable period of time from the expected date of surgery, or within such period as may be provided by company rules and regulations or by collective bargaining agreement.

Prior application for leave shall not be necessary in cases requiring emergency surgical procedure, provided that the employer shall be notified verbally or in written form within a reasonable period of time and provided further that after the surgery or appropriate recuperating period, the female employee shall immediately file her application using the prescribed form.

Section 4. Availment. -Special leave benefits shall be granted to the qualified employee after she has undergone surgery, without prejudice to an employer allowing an employee to receive her pay before or during the surgery.

Section 5. Benefits. - The employee is entitled to full pay for two months based on her gross monthly compensation. Gross monthly compensation refers to the monthly basic pay plus mandatory allowances fixed by the regional wage boards.

Section 6. Non-commutation of benefits. - This special leave shall be non-cumulative and non-convertible to cash unless otherwise provided by a collective bargaining agreement (CBA).

Section 7. Enforcement and monitoring. - The Labor Inspectorate of the DOLE Regional Offices shall be responsible for the enforcement and monitoring of this Guidelines.

Section 8. Transitory Provision.- Subject to the provisions of Section 2 herein, female employees who have taken a leave of absence following surgery for gynecological disorder on or after 15 September 2009 are entitled to the special leave benefits for women.

Section 9. Effectivity.-This Guidelines shall take effect fifteen (15) days after its publication in a newspaper of general circulation.

Manila, Philippines, March 11, 2011.


ROSALINDA DIMAPILIS-BALDOZ
Secretary